

THE DENTAL GROWTH PLAYBOOK

More New Patients, **Every** **Single Month.**

The exact growth system we use to help dental practices generate demand, capture enquiries, book appointments, reduce friction, and measure what actually produces kept patients.

100+
DENTAL PRACTICES

\$380K
PEAK MONTHLY SALES

2021
SINCE

CONTENTS

What's Inside

01	Introduction - Why this playbook exists	03	15	Reputation & review management	18
02	Track record & what we have learned	04	16	The lead capture layer	19
03	The new patient journey, mapped	05	17	Lead forms that reduce friction	20
04	Channel sequencing: what to build first	06	18	Online appointment booking models	21
05	Google Ads foundations for dental practices	07	19	Scheduling software & booking apps	22
06	Dental Ads non-negotiables & budgeting	08	20	CRM pipelines & lead ownership	23
07	Local SEO & Google Business Profile	10	21	WhatsApp automation	24
08	Citations, service areas & local authority	11	22	Missed-call recovery & speed-to-lead	25
09	Websites that convert clicks into chairs	12	23	AI-assisted intake & follow-up	26
10	Procedure landing pages that convert	13	24	Reminders, confirmations & no-show reduction	27
11	Content & topical authority SEO	14	25	Full-funnel tracking & ROI	28
12	How SEO compounds over time	15	26	Multi-location scaling & system architecture	29
13	Social & Meta Ads for patient acquisition	16	27	Your 30-day quick-start checklist	30
14	Dental creative that actually earns attention	17	28	Get your free practice growth audit	31

INTRODUCTION

Why We Wrote This Playbook

Most dental practices do not have a marketing problem. They have a systems problem - and it costs them chairs every week.

We regularly see practices running paid search without call tracking, publishing content that does not match patient intent, or generating leads with no reliable process for getting those leads into the schedule. The issue is rarely effort. It is usually sequencing, ownership, and friction.

This playbook maps the entire growth path - from discovery to trust, from lead capture to appointment booking, from follow-up to attendance, and finally to revenue attribution.

Who this is for: Practice owners, office managers, and marketing leads who want a clear map of how digital demand becomes booked and attended patients.

How to use this book

Each chapter stands alone, but the order matters. If you are starting from scratch, move through the playbook in sequence. Fix conversion before buying more traffic. Fix booking before scaling lead volume. Fix tracking before judging a channel.

The chapters after paid media and SEO are intentionally operational. Modern dental growth is no longer just an advertising problem; it is also an intake, scheduling, automation, and measurement problem.

TRACK RECORD

What We've Done for Practices Like Yours

Bixeltek is a Google Partner-certified digital marketing and technology agency, and dental has been our deepest concentration since 2021. Across that time, BixDental has managed digital presence and paid acquisition for more than 100 dental clinics across multiple markets.

100+ DENTAL CLINICS	\$0 → \$380K MONTHLY SALES GROWTH RANGE	3 yrs TO MULTI-LOCATION SCALE
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We have worked with brand-new practices from a standing start and with established clinics already producing meaningful monthly revenue. The recurring lesson is that acquisition performs best when the rest of the system is designed to receive it.

This is not a "we ran some ads" story. The strongest outcomes come from a connected stack: intent-based campaigns, local search visibility, conversion-focused pages, low-friction lead capture, scheduling workflows, follow-up, and attribution.

What this means for this book

The frameworks in the following pages come from that hands-on operating experience. The new chapters on booking, CRM, WhatsApp, AI-assisted intake, reminders, and missed-call recovery are here because those systems increasingly determine whether a marketing lead becomes an actual patient.

CHAPTER 01

The New Patient Journey, Mapped

Before you spend another rupee or dollar on marketing, you need to understand the path a patient actually takes. It is rarely one click and one booking.

01 TRIGGER

Pain or need

Toothache, chipped tooth, cleaning reminder, cosmetic goal, new insurance, move to a new area.



02 DISCOVERY

Search & social

Google Search, Maps, Meta, referral link, local content.



03 TRUST

Proof

Reviews, website, before/after work, team, credentials, pricing clarity.



04 CONVERSION

Lead or booking

Call, form, WhatsApp, booking widget, consultation request.

The journey does not end at the lead

05 RESPONSE

Speed & ownership

Who responds, how fast, and through which channel?



06 BOOKING

Schedule access

Request, real-time slot, hybrid confirmation, intake.



07 ATTEND

Confirmation

Reminders, reschedule links, waitlist, pre-visit forms.



08 VALUE

Revenue & recall

Production, reactivation, reviews, referrals, lifetime value.

The uncomfortable truth: many practices invest only in Discovery. The biggest leaks often happen later - after the patient already showed intent.

STRATEGY

Why We Sequence the Stack This Way

Because the patient journey has multiple failure points, the order in which you build the stack matters more than the number of channels you buy.

Stage	Focus	Why it comes here
Month 1	Google Business Profile + website conversion fixes	Fix trust and conversion before paying for more traffic.
Month 1-2	Tracking + Google Ads	Create a measurable source of high-intent demand.
Month 2-4	Local & topical SEO	Build a compounding organic acquisition layer.
Month 2+	Lead capture + booking workflows	Reduce the gap between inquiry and confirmed appointment.
Month 3+	Meta / retargeting / nurture	Build recall and re-engage prospects who did not convert.
Ongoing	Reviews, reminders, reactivation, reporting	Strengthen trust, attendance, retention, and decision-making.

Quick self-check: Can you name the source, response time, booking status, attendance status, and production value of your last five new-patient enquiries? If not, the system still has blind spots.

Our operating principle

Do not scale a leak. More traffic into a weak intake process simply creates more expensive missed opportunities. Fix the downstream workflow before raising the upstream budget.

CHAPTER 02

Google Ads Foundations for Dental Practices

Google Ads should mirror the way patients search - not the way your service menu is organized.

Google Ads is one of the fastest controllable ways to capture high-intent dental demand. It is also one of the fastest ways to waste budget when campaigns chase cheap clicks instead of patients who are ready to call, book, or request treatment.

Structure around non-brand patient intent

- **Emergency & pain-driven:** emergency dentist, toothache, broken tooth, same-day care.
- **High-value restorative & cosmetic:** dental implants, veneers, Invisalign, full-mouth rehabilitation.
- **General & new-patient:** dentist near me, dentist accepting new patients, cleaning and exam.
- **Location + procedure combinations:** searches where treatment need and geography are both obvious.

Our approach to brand keywords: BixDental does not use branded-keyword campaigns as a core patient-acquisition strategy. Growth budget is focused on non-brand, high-intent searches that can introduce the practice to patients who are actively choosing a provider.

Set the right expectation for lead cost

For competitive Canadian and U.S. dental markets, a common working range for a qualified Google Ads lead is approximately **\$70-\$125**, depending on the region, procedure, competition, landing-page strength, and tracking quality. A lower CPL is not automatically better: high-intent dental leads tend to cost more because they are much closer to making a booking decision.

Do not optimize for cheap clicks

A \$3 click that never becomes a patient is not more efficient than a \$15 click that produces a booked consultation.

Judge by booked patients

Track lead-to-booked and booked-to-attended rates by campaign and treatment line.

Expect variation

Emergency, general, implant, orthodontic, and cosmetic campaigns will have different economics.

What we watch weekly

- Search terms and negatives
- Cost per qualified lead
- Calls vs. forms vs. booking starts

What we watch monthly

- Cost per booked appointment
- Cost per attended appointment
- Procedure mix and patient value

PAID ACQUISITION

Google Ads: Budget, Common Mistakes & Display

A healthy starting budget

For a dental clinic entering Google Ads with proper tracking and conversion infrastructure, a practical starting point is around **\$2,000 in month one**. Once the campaign structure, search terms, call handling, and landing-page path begin to stabilize, increasing toward **\$3,000 in month two** usually gives the account more room to learn and scale.

\$2,000

MONTH 1 - CONTROLLED LAUNCH

Validate search intent, negatives, geography, conversion tracking, call quality, and the lead-handling process.

\$3,000

MONTH 2 - STABILIZE & SCALE

Increase spend after the account has enough clean data to identify which procedures, keywords, and locations are producing real bookings.

Common mistakes that quietly burn budget

Broad, mixed intent

Combining emergency, implants, cleaning, jobs, research, and informational traffic in the same campaign or ad group.

Homepage as the landing page

Paying for procedure-specific intent and then forcing the patient to search the website for the next step.

Optimizing to form volume

Counting every form or call as equal instead of checking whether it became a booked and attended patient.

No call attribution

Dental leads often convert by phone. Without campaign-level call tracking, optimization is incomplete.

Weak negative-keyword hygiene

Allowing job seekers, students, free-care searches, training queries, or irrelevant services to consume budget.

Changing too much too quickly

Daily bid and targeting changes can destroy the learning needed to judge a campaign fairly.

Budget discipline: month-one budget should buy enough clean search data to make decisions. Scaling should follow evidence - not impatience. If the leads are poor, fix targeting and conversion before adding spend.

PAID ACQUISITION

Display Ads: Where Cheap Clicks Fit

Display is inexpensive because the user is browsing - not actively searching for a dentist.

Display campaigns often produce impressive-looking CPCs, reach, and impression counts. The problem is intent. A person searching "**emergency dentist near me**" is expressing an immediate need; a person seeing a banner while reading the news is not. That is why Display should not be evaluated as if it were Search.

Search Ads	Display Ads
User is actively searching for dental care	User is browsing websites, apps, or content
Higher CPC, stronger commercial intent	Lower CPC, weaker direct-response intent
Primary role: patient acquisition	Primary role: awareness and remarketing
Judge by booked / attended patients	Judge by assisted conversions, reach, and remarketing impact

Where Display can work well

- **Remarketing:** stay visible to recent website visitors who researched a treatment but did not contact the clinic.
- **High-value treatment reinforcement:** support implants, Invisalign, cosmetic dentistry, or full-mouth cases with repeated brand exposure.
- **New-location or offer awareness:** build familiarity around a clinic opening, new service, or limited campaign.
- **Creative testing:** learn which messages and treatment hooks attract attention before scaling broader awareness.

What not to do

Weak use

- Replacing Search with Display because clicks are cheaper
- Broad audiences with no frequency control
- Sending all traffic to the homepage
- Reporting impressions as patient growth

Better use

- Remarket known website visitors
- Use treatment-specific creative
- Cap frequency and watch assisted conversions
- Keep Search as the primary acquisition engine

BixDental principle: cheap traffic is not the goal. Display earns its place when it supports a functioning Search, landing-page, tracking, and booking system - not when it substitutes for one.

Use Display after

Search campaigns, conversion tracking, landing pages, and lead handling are stable.

Measure with context

Look for view-through / assisted impact and returning-user conversion, not just last-click leads.

CHAPTER 03

Local SEO & the Google Business Profile

Your local presence needs to earn both visibility and trust.

For local searches such as "dentist near me" or "dentist in [city]," the Google Map Pack is often one of the first surfaces a patient sees. Your Business Profile is therefore not a directory listing; it is a conversion asset.

Google Business Profile checklist

- ☐ Primary category set accurately, with relevant secondary categories.
- ☐ Every major service listed individually with clear descriptions.
- ☐ Current hours, holiday hours, phone, website, and booking links.
- ☐ Real team, office, technology, and treatment-space photos added consistently.
- ☐ Q&A seeded with real patient questions: insurance, walk-ins, parking, emergencies, financing.
- ☐ Review responses handled professionally and consistently.
- ☐ Name, address, and phone identical across the web.

Common failure point: practices with multiple providers or locations often create duplicate, inconsistent, or poorly governed profiles. Clean location ownership matters before any content or link work begins.

Monthly GBP operating rhythm

- New photos and treatment updates
- Review response and Q&A review
- Hours, services, booking links

Track more than rank

- Calls and website clicks
- Direction requests
- Queries by treatment + location

LOCAL AUTHORITY

Beyond the Profile: Citations, Service Areas & Relevance

1. Citation consistency

Your practice name, address, and phone should match across directories, insurer listings, chamber/business directories, and local profiles. Inconsistency creates avoidable friction for both search engines and patients.

2. Service-area and neighborhood relevance

If a practice attracts patients from several distinct areas, create genuinely useful pages for the most important locations. Avoid thin, duplicated city pages. Each page should answer local questions, link to relevant services, and make the location relationship clear.

3. Internal local authority

LOCAL AUTHORITY STRUCTURE

Google Business Profile

Location page

Implants page

Emergency page

Invisalign page

Local FAQ content

Reviews

Multi-location note: each physical location needs its own profile, location page, tracking, and review-generation process. Shared branding is fine; shared local signals are not.

Local SEO becomes more valuable as the rest of the system improves because stronger visibility feeds a website and booking process that is already designed to convert.

Local relevance signals

Service pages, location pages, internal links, citations, reviews, and the Business Profile should reinforce the same practice identity.

Avoid thin location pages

Each service-area page should answer real patient questions and have unique local value - not a city name swapped into duplicate copy.

CHAPTER 04

Websites That Convert Clicks Into Chairs

A good website does not merely explain the practice. It removes reasons not to act.

We have seen practices reduce acquisition cost without changing the ad account simply by improving the website those campaigns point to.

The conversion fundamentals we fix first

- ❑ **Phone and booking action visible without scrolling** on mobile and desktop.
- ❑ **One primary action per page:** call, request, WhatsApp, or book - not five competing priorities.
- ❑ **Real proof above the fold:** real team, real reviews, real location, relevant credentials.
- ❑ **Fast mobile experience:** patients in pain do not wait patiently for decorative pages to load.
- ❑ **Clear insurance and financing guidance** where cost anxiety is likely to block action.
- ❑ **Lead capture matched to intent:** urgent care may favor click-to-call; elective treatment may need a form or consultation booking.

High friction

- Generic homepage
- Multiple competing CTAs
- Long forms
- Hidden phone number
- Stock imagery

Lower friction

- Intent-matched page
- One clear next step
- Short capture form
- Persistent mobile CTA
- Real proof

Conversion questions to answer

- Can a mobile visitor act in under 10 seconds?
- Is the next step obvious for emergency vs elective care?
- Can a patient book outside office hours?

Website metrics that matter

- Call-click rate
- Form-start to form-complete rate
- Booking-start to confirmed-booking rate

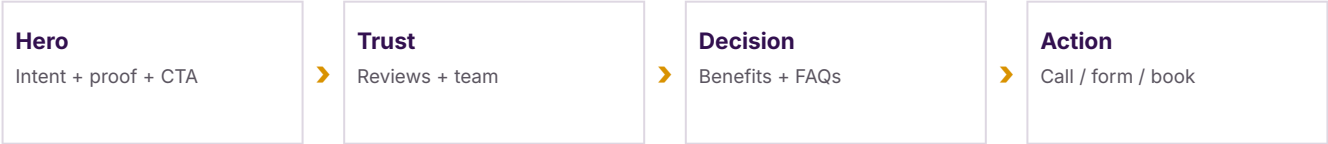
CONVERSION

Procedure Landing Pages That Convert

Your homepage has to serve everyone. A paid-traffic landing page has one job: convert the intent that brought the visitor there.

Section	Purpose
Headline matching search intent	Confirms immediately that the visitor is in the right place.
Real photos / relevant proof	Reduces uncertainty for higher-consideration procedures.
Benefits + common concerns	Answers pain, appearance, time, and outcome questions.
Pricing guidance / financing	Reduces a major objection before the phone call.
Provider credentials	Critical for cosmetic, surgical, and high-value treatment.
Single repeated CTA	Book, call, or request - repeated naturally down the page.

LANDING-PAGE ANATOMY



Treat the page as a living conversion system. Review call clicks, form starts, form completions, booking starts, and bookings - not just pageviews.

Best page-level proof

Real provider photos, relevant reviews, clear treatment explanation, financing guidance, and one visible next step.

Best test sequence

Test headline and CTA first, then form length, proof placement, financing copy, and booking method. Change one major variable at a time.

CHAPTER 05

Content & Topical Authority SEO

Publishing generic dental advice is not a content strategy. Topical authority comes from comprehensively covering the procedures, problems, decisions, and questions your patients actually search.

How a topical content map is built

- 1 **Core service pages** for the procedures you actively want more patients for.
- 2 **Supporting question content** around cost, recovery, pain, candidacy, insurance, and alternatives.
- 3 **Comparison and decision content** for patients evaluating options such as implants vs bridges or Invisalign vs braces.
- 4 **Internal linking** connecting the cluster into a coherent topic around each service line.

EXAMPLE: IMPLANT TOPIC CLUSTER

Dental Implants

Cost

Recovery

Pain

Implant vs bridge

All-on-X

Insurance

Implants in [City]

Editorial rule: write for the decision a patient is trying to make, not for a keyword quota.

Content that supports conversion

Cost, pain, recovery, alternatives, timelines, insurance, candidacy, and comparison queries usually sit closest to a booking decision.

Internal-linking rule

Supporting articles should point patients back toward the relevant service page and the next appropriate action.

ORGANIC GROWTH

Why This Compounds Where Ads Do Not

A paid campaign stops producing the moment you stop buying media. Strong organic assets can keep attracting patients long after publication. That is why SEO is usually a long-term growth layer rather than a short-term replacement for paid search.

Content priorities by practice stage

Stage	Priority
Months 1-2	Core service pages and location fundamentals.
Months 2-5	FAQ, decision, and comparison content around priority procedures.
Months 5+	Neighborhood / service-area depth, secondary services, broader topical expansion.

What compounds

Ranked pages

Each useful page becomes another discovery surface.

Internal authority

Related content strengthens the service pages you care most about.

Trust before contact

Patients arrive having already answered part of their own questions.

Do not pull SEO because paid search is working. The channels solve different time horizons. Search ads capture demand now; organic content can lower dependency on paid acquisition over time.

Do not judge SEO weekly

Measure impressions, ranking distribution, qualified organic leads, and assisted conversions over months - not days.

Compounding advantage

A mature treatment cluster can keep producing visibility after the content-production cost has already been paid.

CHAPTER 06

Social & Meta Ads for Patient Acquisition

Search captures demand that already exists. Meta can create awareness, reinforce familiarity, and re-engage people who were not ready to book the first time.

Two distinct jobs for Meta

Warm retargeting

Website visitors, video viewers, engaged social users, and leads who have not booked. Usually the safest place to start once core tracking is in place.

Cold acquisition

High-value or elective treatments where strong creative, education, and offer structure can create demand before a search happens.

Meta is not Google Search

Google Search	Meta
Captures declared intent	Creates or warms intent
Keyword-led	Creative-led
Urgency often higher	Education and trust often more important
Call / direct booking can work quickly	Lead form + nurture may be more appropriate

Sequencing matters: cold social traffic usually needs an even stronger conversion and follow-up process than high-intent search traffic.

Meta KPI hierarchy

Qualified lead → booked consultation → attended consultation, not impressions or cheap form submissions.

Best starting audience

Warm website visitors and engaged users often make a better first Meta layer than broad cold acquisition.

CREATIVE

Dental Creative That Actually Earns Attention

What tends to earn attention

- **Real team and real office video.** Patients are choosing people and a place, not a graphic template.
- **Procedure education in short form.** One question, one answer, one next step.
- **Before/after or outcome-led creative** where appropriate, permitted, and supported by patient consent.
- **Clear offers and financial clarity** without turning the practice into a discount brand.
- **Local familiarity:** dentist introductions, office walkthroughs, community involvement, frequently asked questions.

A simple creative matrix

Angle	Example	Best next step
Problem	"Missing a tooth? Here are your options."	Learn / consult
Proof	Patient story, review, provider explanation.	Book consultation
Offer	New-patient exam, financing, limited consultation availability.	Lead form / booking
Education	"Does a root canal hurt?"	Watch / visit service page

Creative rule: the ad should pre-sell the next step. If the next step is a long form or a confusing website, the creative cannot rescue the funnel.

Creative operating rhythm

Build a repeatable library of short team videos, FAQs, treatment education, office proof, and compliant before/after assets.

Refresh before fatigue

When frequency rises and response falls, rotate hooks and formats before simply increasing budget.

CHAPTER 07

Reputation & Review Management

Reviews are not a side project. They influence local visibility, ad conversion, website trust, and the final choice between similar practices.

A simple, repeatable review system

- 1 Ask at the moment of highest satisfaction, while the positive experience is fresh.
- 2 Make it one tap: direct link, QR code, text message, or post-visit workflow.
- 3 Respond to positive and negative reviews professionally and promptly.
- 4 Route service recovery to a private human conversation when an issue needs resolution.
- 5 Track review velocity by location, not just total count.

REVIEW LOOP

Visit

Good experience



Request

Automated prompt



Review

Public proof



Response

Practice replies



Insight

Use feedback

Compounding effect: a healthy review engine strengthens almost every other acquisition channel because it improves trust at the moment of choice.

Review system ownership

Assign one person to review velocity, response time, unresolved complaints, and monthly review-generation activity.

What patients notice

Recent reviews, response quality, recurring themes, and whether the practice appears attentive when something goes wrong.

CHAPTER 08

The Lead Capture Layer

The bridge between marketing activity and a real patient conversation.

Generating interest is only half the job. The lead capture layer decides whether that interest becomes an identifiable person the practice can respond to.

Choose the capture method by intent

Patient situation	Best capture path	Why
Emergency / pain	Click-to-call + missed-call recovery	Urgency is high; speed matters more than education.
General new patient	Short form or online booking	Simple intent; patients often want convenience.
Implants / Invisalign / cosmetic	Lead form + consult booking + nurture	Higher consideration and more questions before commitment.
After-hours enquiry	Online booking / WhatsApp / automated intake	Do not force the patient to wait for office hours just to express intent.

What a good capture layer does

Identifies intent Why is the patient contacting the practice?	Captures the minimum Enough information to continue the conversation.	Creates a next step Call, booking, message, or nurture - never a dead-end thank-you page.
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Key idea: do not ask for information merely because your CRM has a field for it. Every extra field is friction unless it helps the next step.

Capture every pathway Phone, short form, booking widget, WhatsApp, and Meta instant lead should all create a trackable lead record.	One owner per lead Every enquiry needs a clear person or queue responsible for response, booking, and follow-up.
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LEAD CAPTURE

Lead Forms That Reduce Friction

Overloaded form

- Full name
- Email
- Phone
- Date of birth
- Insurance
- Preferred provider
- Preferred day
- Preferred time
- Address
- Message

Lower-friction first step

- Name
- Mobile number
- What can we help you with?
- Optional: preferred contact method

Then move the patient forward

STEP 1

Capture

Minimum viable details.



STEP 2

Qualify lightly

Treatment / urgency / location.



STEP 3

Offer action

Book now, choose callback, or message.



STEP 4

Enrich later

Insurance and clinical intake after booking.

Form design rules

- ☐ Use one column on mobile.
- ☐ Make error messages specific and immediate.
- ☐ Do not force account creation just to request an appointment.
- ☐ Confirm what happens next and how quickly the practice will respond.
- ☐ Track form starts, partial completion, submits, and downstream bookings separately.

Important: intake forms and marketing lead forms are not the same thing. Collect clinical and administrative detail after the patient has committed to the appointment wherever possible.

Form principle

Ask only for information required to move the patient to the next step. Collect the rest after contact or booking.

Measure friction

Track form starts, completions, validation errors, mobile completion, and how many forms become appointments.

CHAPTER 09

Online Appointment Booking Models

"Book online" can mean very different things. Choose the model based on how complex the schedule is and how much human validation the appointment requires.

Three booking models

1. Appointment request Patient submits preferred times. Team reviews and confirms manually.	2. Real-time booking Patient sees actual availability and selects a slot immediately.	3. Hybrid booking Simple visit types book instantly; complex cases trigger a confirmation workflow.
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Model	Best for	Main risk
Request	Complex schedules, specialty triage	Slow follow-up creates abandonment
Real-time	New-patient exams, hygiene, simple consultations	Poor rules can create operational chaos
Hybrid	Mixed practices	Needs clear appointment logic and staff ownership

Booking rules to define before launch

- ☐ Appointment types and durations
- ☐ Providers and operatories
- ☐ New vs existing patient rules
- ☐ Lead time and same-day limits
- ☐ Insurance / deposit requirements where applicable
- ☐ Rescheduling and cancellation logic

Modern standard: the patient should not need to make a phone call just to discover whether the practice has availability.

Booking setup checklist • Correct appointment types • Buffer and provider rules • Real availability and confirmations	Do not overexpose the schedule Use rules to protect procedure time, new-patient slots, emergencies, and staff workflow.
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TECHNOLOGY

Scheduling Software & Booking Apps

There is no single "best" booking app for every practice. Evaluate the category first, then the vendor.

Category	What to look for	Examples to evaluate
PMS-connected online scheduling	Real-time availability, provider rules, writeback, appointment types	NexHealth; Dental Intelligence Online Scheduling
Patient communication platforms	Scheduling + reminders + two-way messaging + forms	Weave; Solutionreach
CRM-led consultation funnels	Lead capture, pipeline, nurture, calendar handoff	Useful for elective / high-value treatment campaigns

Capabilities worth prioritizing

- **Real-time or frequently synchronized availability** to reduce double-booking and manual confirmation.
- **Website and Google booking entry points** so patients can act where intent happens.
- **Appointment-type rules** for provider, duration, operatory, and service eligibility.
- **Automated reminders, confirmations, and rescheduling** to reduce staff workload after booking.
- **Waitlist / quick-fill features** to refill cancellations.

Examples: NexHealth describes real-time online booking across website, social, text, email, and Google. Dental Intelligence positions its former LocalMed product as Online Scheduling with Reserve with Google and quick-fill tools. Weave and Solutionreach both combine patient communication with scheduling and reminders.

Vendor examples are illustrative. Verify compatibility with your practice management system, local privacy requirements, pricing, and workflow before implementation.

Evaluate software by workflow

Integration depth, schedule accuracy, two-way sync, confirmations, analytics, and patient experience matter more than the prettiest widget.

Implementation test

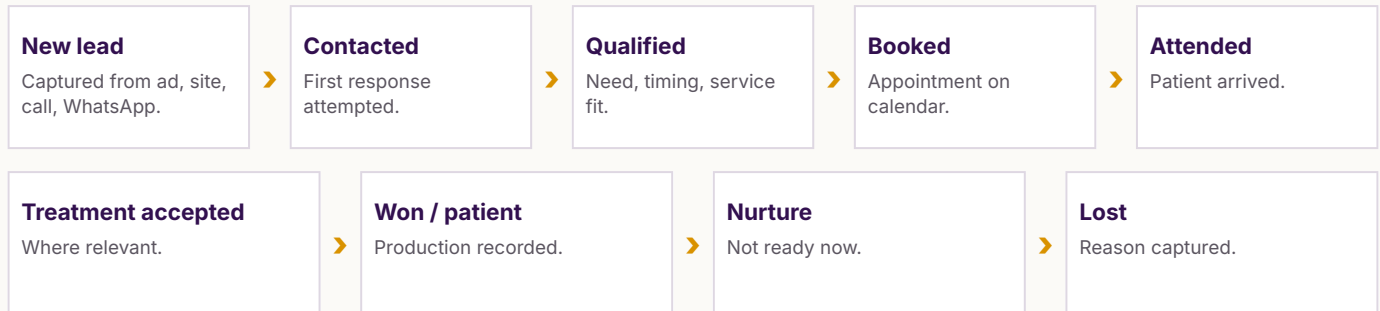
Book a real test appointment from mobile after hours and verify every notification, CRM update, and calendar change.

CHAPTER 10

CRM Pipelines & Lead Ownership

Once a lead exists, somebody must own it. A CRM is useful only if it makes that ownership visible.

A simple dental lead pipeline



Minimum CRM fields

SOURCE	CAMPAIGN	TREATMENT	OWNER	RESPONSE TIME	BOOKING DATE	ATTENDANCE	LOST REASON
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Operational rule: every lead should have an owner, a current stage, and a next action. A lead sitting in "New" with no task is not a lead-management system.

Minimum CRM fields

Source, treatment interest, lead owner, current stage, last touch, next task, booking date, and lost reason.

Why lost reasons matter

They reveal whether the true problem is price, insurance, no availability, no response, poor follow-up, or low-quality acquisition.

PATIENT MESSAGING

WhatsApp Automation Without the Friction

In markets where WhatsApp is a normal customer-service channel, it can remove friction between intent and booking - especially for after-hours enquiries, younger patients, and people who prefer messaging over calls.

EXAMPLE WHATSAPP FLOW

1. Entry

Website, Meta ad, QR, missed-call text.



2. Intent

Emergency, implants, cleaning, Invisalign, other.



3. Next step

Book slot, request callback, ask a question.



4. Handoff

Human takes over when needed.

Good automation

- ☐ Confirms the practice received the enquiry immediately.
- ☐ Asks one useful question at a time.
- ☐ Offers clear paths rather than an open-ended chatbot maze.
- ☐ Passes the conversation to a staff member for clinical judgment, complaints, or complexity.
- ☐ Logs the source and outcome into the CRM where possible.

Do not automate consent away. Messaging workflows must respect local privacy, marketing-consent, and patient-communication requirements. Keep clinical advice and sensitive decisions with qualified staff.

WhatsApp works best when

The patient can ask a simple question, choose a route, book or request a callback, and quickly reach a human.

Avoid chatbot dead ends

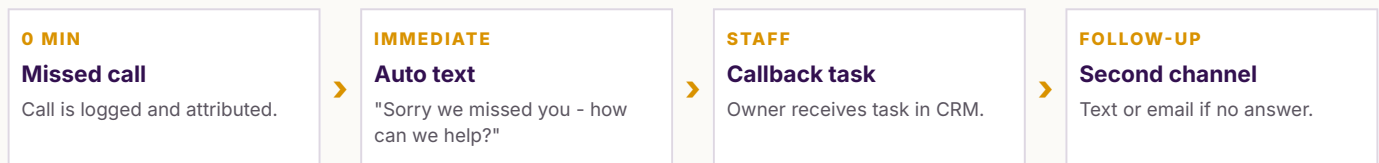
Do not force patients through ten automated questions before giving them a clear human handoff.

CHAPTER 11

Missed-Call Recovery & Speed-to-Lead

A missed call is not just a phone event. It is a high-intent lead that may be calling the next practice within minutes.

The missed-call recovery workflow



Speed-to-lead principles

- ☐ Define a response SLA for working hours.
- ☐ Separate true new-patient calls from existing-patient service calls in reporting.
- ☐ Use missed-call text recovery so the patient can continue asynchronously.
- ☐ Create after-hours paths for emergencies and online booking.
- ☐ Track first-response time by staff member and source.

Why this matters: patient communication vendors such as Weave explicitly offer missed-call text workflows because a lost call can become lost business. The system should assume that unanswered intent decays quickly.

Missed-call KPI

Track how many missed new-patient calls receive a response, how quickly, and how many are recovered into appointments.

After-hours opportunity

Let high-intent patients leave structured information or book appropriate slots instead of waiting until the next business day.

CHAPTER 12

AI-Assisted Intake & Follow-Up

Use AI to remove repetitive work - not to remove human accountability.

AI is useful when it removes repetitive friction. It is not a substitute for clinical judgment, empathy, or accountable front-desk ownership.

Good uses for AI-assisted workflows

Before booking

Answer repetitive non-clinical FAQs, identify intent, capture basic information, propose next steps.

During handoff

Summarize conversations, tag treatment interest, create CRM tasks, route to the right team member.

After booking

Trigger reminders, form links, directions, preparation instructions, and rescheduling options.

After missed conversion

Re-engage abandoned enquiries, ask whether the patient still needs help, and surface warm leads to staff.

THE OPERATING PRINCIPLE

AI handles repetition

Routing, FAQ, summaries, triggers.

➤

Team handles judgment

Clinical questions, complaints, treatment decisions, exceptions.

➤

Patient gets less friction

Faster, clearer next steps.

Guardrail: keep sensitive health information, clinical advice, and regulated communications within approved systems and policies. "AI-enabled" should never mean "uncontrolled."

AI should create momentum

Classify intent, summarize context, propose next actions, and keep staff from re-reading long conversations.

Human escalation triggers

Clinical advice, complaints, medication questions, payment disputes, emergencies, or uncertainty should route to staff.

PATIENT EXPERIENCE

Reminders, Confirmations & No-Show Reduction

Booking is not the finish line. A booked patient who forgets, cancels late, or cannot reschedule easily still creates an empty chair.

Build an attendance system

- **Confirmation immediately after booking:** date, time, location, provider, next steps.
- **Reminder cadence:** appropriate advance reminders through the patient's preferred channel.
- **Two-way confirmation:** make it easy to confirm, cancel, or request a change.
- **Self-service rescheduling:** reduce phone tag when the patient cannot attend.
- **Waitlist / quick fill:** notify suitable patients when a cancellation opens.
- **Recall / reactivation:** prompt unscheduled patients who are due back.

Why integrated communication helps

Platforms such as Weave and Solutionreach combine reminders with two-way messaging and rescheduling workflows, while Dental Intelligence includes quick-fill scheduling. The exact product matters less than ensuring the workflow is automated, visible, and connected to the schedule.

Measure attendance, not just bookings. Your true funnel is lead → booked → confirmed → attended. Each transition deserves its own conversion rate.

Attendance metrics

Confirmation rate, reschedule rate, cancellation lead time, no-show rate, and waitlist-fill rate.

Patient-friendly automation

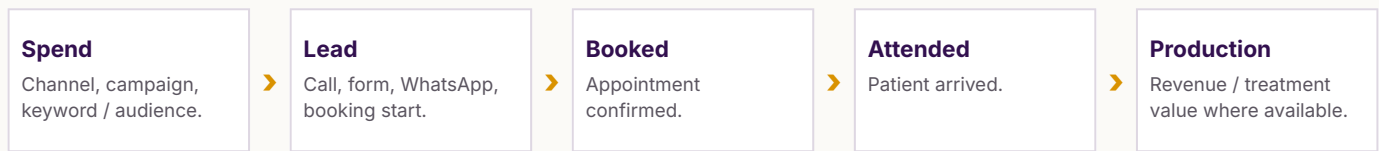
Every reminder should make the next action easy: confirm, reschedule, cancel, call, or get directions.

CHAPTER 13

Full-Funnel Tracking, Attribution & ROI

If you cannot tell which channel produced which booked and attended patient, marketing decisions remain guesswork.

The minimum full-funnel tracking model



Metric	What it tells you
Cost per lead	How efficiently a channel creates identifiable intent.
Lead-to-booked rate	How well intake and scheduling convert interest.
Booked-to-attended rate	How well reminders and patient experience protect the schedule.
Cost per kept appointment	The clearest acquisition-efficiency metric for most practices.
Production by source	Which channels create economic value, not just volume.

The metric that matters most: raw leads are not the destination. A channel should be judged by the patients it actually helps put in the chair.

Monthly management view

Spend → qualified leads → booked → attended → production by channel and treatment line.

Do not hide operational leakage

If leads are healthy but booking is weak, the problem may be response speed, availability, scripting, or follow-up - not advertising.

CHAPTER 14

Multi-Location Scaling & System Architecture

Scaling to additional locations is easier when the first location is already running on a repeatable system.

What changes when you add locations

- **Separate local presence:** Business Profile, location page, review flow, phone tracking, and local reporting.
- **Shared brand layer:** creative standards, offer logic, messaging principles, and central governance.
- **Location-specific acquisition:** budgets, search terms, radius, availability, and procedure priorities.
- **Centralized CRM and reporting:** one operating view with location-level ownership.

The modern dental growth stack

Demand Google Ads, SEO, GBP, Meta, referrals.	Conversion Landing pages, calls, forms, WhatsApp.	Booking Online scheduling, callback, hybrid rules.
CRM Lead owner, stage, tasks, nurture, lost reasons.	Attendance Confirmations, reminders, reschedule, waitlist.	Measurement Source → lead → booked → attended → production.

Scaling principle: a second location should be a repeat of a proven operating system, not a fresh experiment built from scratch.

Scale the operating system

Standardize reporting, CRM stages, automation rules, landing-page templates, and review workflows before adding locations.

Keep local control

Each location still needs its own availability, geography, GBP, reviews, staff ownership, and campaign economics.

CHAPTER 15

Your 30-Day Quick-Start Checklist

If you take nothing else from this book, use this sequence. It fixes the highest-friction parts first and gives you a measurable starting point.

Week 1 - Fix the foundation

- ☐ Audit Google Business Profile, categories, services, photos, hours, and booking links.
- ☐ Confirm NAP consistency across the site and major directories.
- ☐ Add persistent click-to-call and one clear booking action to priority pages.
- ☐ Map the current patient journey from first click to attended appointment.

Week 2 - Tracking + capture

- ☐ Install call and form tracking before increasing paid spend.
- ☐ Simplify lead forms and define response ownership.
- ☐ Create a CRM pipeline with stages from New Lead to Attended.

Week 3 - Booking + follow-up

- ☐ Choose request, real-time, or hybrid online booking.
- ☐ Set missed-call recovery and after-hours capture.
- ☐ Implement reminders, confirmation, and rescheduling workflows.

Week 4 - Acquisition + optimization

- ☐ Launch or restructure intent-based Google Ads.
- ☐ Improve the top 2-3 procedure landing pages.
- ☐ Turn on review requests and retargeting where appropriate.
- ☐ Review lead-to-booked, booked-to-attended, and cost-per-kept-appointment.

At the end of 30 days: you should be able to answer four questions with data: Where are leads coming from? How quickly are they handled? What percentage book and attend? What does a kept patient cost by channel?

NO OBLIGATION · PRACTICAL RECOMMENDATIONS

Claim Your **\$250** Dental Growth Audit **Free.**

Get an expert review of the digital system that brings patients from search to booked appointment - plus a private 30-minute one-on-one consultation with BixDental's Google Ads-certified consultants with specific dental-clinic experience across Canada and the USA.

Your complimentary audit includes

- **Google Ads audit:** campaign structure, keywords, search terms, location targeting, conversion tracking, CPL and booking quality.
- **Website & landing-page audit:** speed, mobile usability, CTAs, forms, booking flow, trust signals and conversion friction.
- **SEO & local visibility audit:** Google Business Profile, service-page coverage, local SEO, content gaps and search visibility.
- **Lead-to-booking audit:** forms, appointment scheduling, call handling, WhatsApp, follow-up, reminders and CRM opportunities.
- **30-minute one-on-one consultation:** prioritized recommendations and a practical next-step plan - yours to keep whether or not you work with us.

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Bixeltek.com

zee@bixeltekglobal.com · +1 (437) 525-2301